NIHR | National Institute for Health and Care Research

Summary

Reference Number	NIHR305215
Lead Applicant	Dr Dawn Cutler
Research Title	How Does a Therapeutic Milieu Help Children who Need Treatment in Hospital for Mental Health Difficulties?
Plain English Summary	**Background** Around 3,000 children and young people need treatment in hospitals for mental health difficulties each year in the UK. Children are treated in hospital when community teams cannot find a way to help and families may be finding it difficult to keep them safe at home. When children stay on a mental health ward they have a team looking after them day and night. This team of people ensure that the whole environment around the child is helping them. The ward environment **is** the treatment – this is called a 'therapeutic milieu'. Staying in hospital for treatment is a huge commitment for children and their families with emotional, practical and financial pressures. Given how hard it is to be in hospital, it is important for us to get treatment right. **Aims** We want to know more about how change happens when children stay in hospital for treatment for mental health difficulties. In particular, we want to know how the therapeutic milieu in hospital helps (or does not help). This will help us know how to improve the treatment process and understand when, how and why this type of treatment works. This supports children to get well enough to go home as quickly as possible, with their mental health improved. **Plans for the Project** The project will have three parts which link to each other. Part 1 We will look at what existing studies tell us about the experiences of children and parents/carers when a child is in hospital for mental health treatment. Part 2 All staff who work on the six mental health wards for children under 13 years old in England will be invited to complete a survey, to understand how each ward works and how they compare to each other. This will be the first time there has been a national survey of practice for children's units. Part 3 We will use an approach to observing and understanding how things work called ethnography. This will take place at two hospitals in different parts of England. I (DC) will observe how staff work in real life to understand how the ward works at different times of day and night over many months. This will help us learn in detail the processes that staff actually take to make a therapeutic milieu. As part of this we will interview staff, children and the parents (or carers) of the children on the wards. We will ask them about how the environment of the ward helps and does not help. Children and parents will speak to the researcher twice – once during their stay in hospital and once after they have left. We will record what they say and then summarise and interpret what they have told us. **Public and Patient Involvement** A parent told us that a member of the ward team treating their child had said to them about treatment 'we don't know how it works but it does'. This gave us the idea for the study. Children staying on a children's mental health ward helped develop the ideas for this project and write this summary. Their parents also helped. People who stayed on children's mental health wards in hospital when they were children have also contributed. These people are all experts due to their experiences. There will continue to be a group of experts by experience who support the researcher at all stages of the project. **Sharing the Results** We will create an animation to share the results with children and parents. We will use the results to improve the way we work in inpatient mental health services nationally.

Our purpose for collecting this data is to communicate with you about your application and have the necessary information to evaluate you for a grant. The data we collect here is collected in the public interest. Information provided here may be subject to Freedom of Information requests.

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The NIHR privacy policy includes further information including ways we may use your data, our contact details and details on your individual rights regarding how your data is used. Your data may be shared across the NIHR, including with other coordinating centres, to allow the application to be managed and for statistical analysis, and with external grant reviewers as part of the process for managing the allocation of a grant. This notice is under constant review and will be updated and / or revised based on that review as appropriate.

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1. Application Summary Information

Host organisation (which will administer any award)
Great Ormond Street Hospital For Children NHS Foundation Trust

Partner organisation	University College London

Research Title	How Does a Therapeutic Milieu Help Children who Need Treatment in Hospital for Mental Health Difficulties?

Partnership Fellowship Please select the relevant Partnership Fellowship.
N/A

Proposed WTE Please select which WTE option you would prefer
0.80

Duration (months) The duration for your award is based on your chosen WTE
45

Proposed start date if grant awarded	01/09/2025

Research Type	Qualitative study

2. Applicant CV

A CV for the Applicant is a mandatory component of this application; please ensure this is completed fully.
Please note that some of the CV questions are automatically completed from information you have provided in the 'Update CV' section of the 'Manage My Details' page. These can be updated via the ARAMIS Home Page or by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Degrees and Professional Qualifications

From	To	Qualification	Subject	Country	Institution	Class
09/2014	06/2015	Post-Graduate Diploma	Applied Systemic Theory	United Kingdom (England)	University of East London	Merit
10/2004	09/2007	Doctorate in Clinical Psychology (DClinPsy)	Professional qualification in clinical psychology	United Kingdom	University College London	Pass
09/2000	06/2001	Special Student / Herchel Smith Scholar	I studied a diverse range of subjects at both undergraduate and postgraduate level	United States	Harvard University	NA
09/1997	06/2000	MA	Natural Sciences (Experimental Psychology)	United Kingdom	University of Cambridge	1st

Present and previous positions
(including from/to dates, job title and employer)

From	To	Position	Organisation	WTE % Research
09/2023	08/2024	Pre-Doctoral Clinical Academic Fellow (0.4 FTE)	Great Ormond Street Hospital For Children NHS Foundation Trust	40
09/2022	03/2023	Research Internship (0.2 FTE)	Great Ormond Street Hospital For Children NHS Foundation Trust	20
10/2021	-	Clinical Psychologist (0.1 FTE)	Power the Fight (Charity Empowering Communities to End Youth Violence)	2
02/2021	-	Principal Clinical Psychologist - Mildred Creak Unit - Inpatient CAMHS (0.5 FTE)	Great Ormond Street Hospital For Children NHS Foundation Trust	5
11/2016	06/2021	Principal Clinical Psychologist - Paediatric Psychology - Gastroenterology (0.8 FTE)	Great Ormond Street Hospital For Children NHS Foundation Trust	5
08/2010	11/2016	Locality Lead - Clinical Psychologist	First Steps Early Intervention and Community Psychology Service - Hackney CAMHS	5
11/2007	08/2010	Clinical Psychologist	Islington Community CAMHS	5
10/2004	09/2007	Trainee Clinical Psychologist	Camden and Islington NHS Foundation Trust	25
06/2003	08/2004	Research Assistant	Dementia Research Centre - Institute of Neurology - University College London	90
10/2001	06/2003	Associate Consultant	L.E.K. Consulting (Strategic Business Consultancy) - London	0

Research Grants held

Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
NIHR GOSH BRC and GOSH R&I Internship Award	National Institute of Health Research (NIHR) Great Ormond Street Hospital Biomedical Research Centre (GOSH BRC) and GOSH Directorate of Research and Innovation (GOSH R&I) award for an internship position. I applied for and was awarded this grant via a competitive process. It supported my salary for one day per week.	9764	NIHR GOSH BRC and Great Ormond Street Hospital R&I	6	01/10/2022	31/03/2023
HEE/NIHR ICA Pre-doctoral Clinical and Practitioner Academic Fellowship	I applied for the fellowship and managed the awarded grant. The award is supporting my salary two days per week, my training and development during the award and the public and patient involvement work I am completing during this time.	41802	NIHR	12	01/09/2023	31/08/2024

Publication Record

This should display the authors, date, article title and journal name for each publication added. This information can be edited via the “My Research Outputs” page of your ARAMIS account.

Publications
Ridha, BH., Crutch, S., **Cutler, D.**, Frost, C., Knight, W., Barker, S., Epie, N., Warrington, EK., Kukkastenvehmas, R., Douglas, J. and Rossor, MN.. (2018) ‘A double-blind placebo-controlled cross-over clinical trial of DONepezil In Posterior cortical atrophy due to underlying Alzheimer's Disease: DONIPAD study.’, *Alzheimer's research & therapy*, 10(1).
Cutler, D., Murphy, T., Gilmour, J. and Heyman, I.. (2009) ‘The quality of life of young people with Tourette syndrome.’, *Child: care, health and development*, 35(4).
Archer, HA., McFarlane, F., Frost, C., **Cutler, D.**, Fox, NC. and Rossor, MN.. (2007) ‘Symptoms of memory loss as predictors of cognitive impairment?: the use and reliability of memory ratings in a clinic population.’, *Alzheimer disease and associated disorders*, 21(2).
‘Bakshi R, Coulter S, Cutler (2019) D48 The Solihull Approach: an evaluation of staff training on understanding and supporting relationships and emotional communication in a paediatric hospital setting Archives of Disease in Childhood 104:A20.’,
‘Cutler D, Marriott S, McNicholas J (2019) 120 A multi-team approach at GOSH bridging physical and mental health to treat a complex case of medically unexplained symptoms Archives of Disease in Childhood 104:A48.’,
‘Cutler, D (2017) Reciprocity in the Training Relationship - Adapting the Solihull Approach to Training and Consultation. Published in e-ORGANISATIONS & PEOPLE, SUMMER 2017, VOL. 24, NO. 2’,
‘MacKean M, Evans S, Simione S, Cutler, D (2023) 15 The characteristics of patients admitted to a specialist children’s mental health inpatient unit with pervasive arousal withdrawal syndrome. Archives of Disease in Childhood 2023;108:A6.’,
‘Le Pelley M, Cutler D, McLaren I (2000) Retrospective Effects in Human Causality Judgement Proceedings of the

Publication Record

This should display the authors, date, article title and journal name for each publication added. This information can be edited via the “My Research Outputs” page of your ARAMIS account.

Publications
Annual Meeting of the Cognitive Science Society Volume 22’,

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
01/09/2001	Emmanuel College, Cambridge University	Herchel Smith Scholarship - fully funded to attend Harvard University for one year as a special student at the Graduate School of Arts and Sciences.	
01/06/1998	Emmanuel College, University of Cambridge	I was a scholar due to being placed 1st class in part IA (1998) and IB (1999) of my degree.	

ORCID iD 0000-0001-8045-9150

3. Applicant Research Background

Professional Background	Other Health or Care Practitioner

Profession (Other Health or Care Practitioner)	Practitioner Psychologist

Profession (Practitioner Psychologist)	Clinical Psychologist

Do you have current / active professional registration?
This registration must be with one of the **ICA approved regulatory bodies**

Yes

Professional registration number/pin
Please supply a valid professional registration number/pin followed by the regulatory body (eg 12A3456B - NMC or PA12345 - HCPC)

PYL04674 - HCPC

Please describe your research career to date

Research Experience

I am mid-way through my NIHR Pre-doctoral Clinical-Academic Fellowship (PCAF) Bridge Award. Completing this part-time alongside my clinical roles is building a strong foundation for a PhD project by developing good working relationships with my supervisory team and sustaining links with the clinical context where the proposed research will take place.

The bridge fellowship is strengthening and developing my existing research skills. Training in scoping reviews has updated my skills in systematic reviews and I have written the protocol for a scoping review which will be completed summer 2024 and submitted for publication. I have completed training in the methodology of complex intervention research which will support me to develop meaningful research questions and projects in the complex clinical context of inpatient mental health treatment. Training in Patient & Public Involvement (PPI) and linking with the leads for PPI at Great Ormond Street Hospital (GOSH) and Anna Freud has supported the PPI work to develop the proposal and the PPI planning for the DCAF.

In 2023, I was successful in a competitive application for an internship funded by GOSH BRC NIHR to develop my research focus and PCAF application. I have felt privileged to have these opportunities to work as a clinical-academic and to develop an academic career intertwined with my clinical one, addressing research questions developed in my clinical practice.

I have been research lead for Mildred Creak Unit, an inpatient children's mental health unit at GOSH, for the last three years. I have supported engagement with research via a monthly research forum and support for individual projects across disciplines. This engagement has led to us establishing a post for a research assistant in the nursing team, whom I supervise. I have supervised three undergraduate and masters psychology students on theses and projects and all of these have been presented as posters at conferences. I am supporting an MSc student as a co-author to write up and publish their thesis. I am a member of the wider psychology department research committee working on the development of research structure and strategy for the department.

My doctorate in clinical psychology was a combined clinical-academic degree and my thesis was a mixed methods study of quality of life in Tourette Syndrome, written up for successful publication. I shared this research at a UK conference for families and schools about Tourette Syndrome in 2007. My undergraduate thesis exploring a specific aspect of memory formation was the highest marked element of my degree and my supervisor presented

a poster with the findings at relevant conferences.

Prior to clinical psychology training I worked in the Dementia Research Centre within UCL, a national and international centre of excellence for dementia research. I was part of a team working on small and large scale randomised control trials, completing neuropsychological assessments and managing the co-ordination of study visits and data collection within these. I supported vulnerable patients and their carers to participate in these trials.

I have presented in my clinical areas of expertise (e.g. functional symptoms) at teaching days for a range of healthcare professionals including nationwide networks. In 2019 I was an invited speaker at the British Society for Allergy & Clinical Immunology Conference presenting on 'managing the patient's perspective in non-IgE food allergy'.

Clinical Experience

I have worked for 17 years as a qualified psychologist in child and adolescent mental health services (CAMHS) and have been a senior clinician, supervisor and service manager for over 10 years.

NHS: CAMHS and Paediatric Psychology (1994 – current date)

The scientist-practitioner model of psychology means that research is part of my day-to-day clinical practice, for example using Routine Outcome Measures (ROMS) to guide therapeutic work with individuals and to evaluate and develop the service provided. I have a robust knowledge of data collection and analysis in clinical contexts and have led projects which combine clinical and research skills to improve outcomes for patients. For example, I led a needs assessment project which led to the expansion of psychology services within gastroenterology at GOSH utilising data from clinician interviews, analysis of referral patterns, mapping the patient population and administration of ROMS to demonstrate need.

Charity sector: Power the Fight (2021 – current date)

Since its inception in 2019 I have been a member of the advisory group for a charity called 'Power the Fight' working to empower communities to end youth violence. Since 2021 I have worked part-time alongside my NHS roles as a strategic consultant and reflective practice supervisor.

The charity uses co-production as a cornerstone of its approach. I was a co-author for a successful bid for funding from the Mayor of London's Violence Reduction Unit for the 'Therapeutic Intervention for Peace' programme, which responds to the challenges faced by young people and communities through culturally sensitive support for those who are most impacted by youth violence.

Summary

- I have experience in research using both qualitative and quantitative methodologies at different scales and at different stages of the research process. All of my research activity has translated to outputs in appropriate contexts (e.g. posters, funding, publication) raising the profile of research within my clinical teams and providing leadership in this area. This process of translating research into appropriate outputs and sharing these widely is important to me in terms of valuing the contributions of participants and those who support research.
- My clinical experience over many years means I have the core skills and knowledge needed to research complex questions in real-world health contexts and ideas about the questions that are useful to ask.
- A doctoral clinical-academic fellowship would mean I had the training, support and time to complete a PhD, using my clinical experience to develop research which will have an impact on the care provided in the NHS and make a difference to children, families and professionals.
- I hope the proposed PhD would be the foundation of many future research projects in the area of inpatient mental health treatment, enabling me to contribute much needed evidence to this field.

Have you already registered for a PhD?	No

Has this application, or a similar application, previously been submitted to this or any other funding body?
No

Contextual Factors
Please use this question to detail any contextual factors you wish to make the Selection Committee aware of. NIHR wants to know about any circumstances so that they may take them into consideration during the assessment of your application. Contextual factors may include:

- Career breaks due to parental leave, or periods of illness.
- Reduced time spent undertaking research due to a disability or caring responsibilities. This could include any physical or mental difficulty that may have impacted your research career. These are situations that have a significant impact on your ability to undertake research.
- Reduced opportunities to career support e.g. mentorship, and limited opportunities to undertake prior research and training.
- Impact of the COVID-19 pandemic on your research career.

Please also use this section to detail any other factors that may have impacted your research career not listed in the examples provided. The impact on your career to date will be specific to your particular circumstances but could include such impacts as limited opportunities to obtain grant funding or fewer publications. In general terms, mitigating factors should be significant, and relevant.

NIHR acknowledges that you may be reluctant, or uncomfortable disclosing relevant information that is sensitive. However, you should bear in mind that we are unable to take into account factors that you do not disclose. Please be assured that the information provided by you is sensitive and will be treated confidentially and in line with General Data and Protection Regulations (GDPR).

As a newly qualified psychologist I worked in community CAMHS from 2007-16 and focussed on developing as a clinician in this time. During this time, my role included service evaluation and audit but I had less access to research support and opportunities to share these service-related projects. Since moving to GOSH in 2016 I have benefitted from the research structures in a research hospital e.g. support to attend conferences and present posters and advice about accessing funding for research.

Since 2012 I have worked part-time, around four days a week, for personal reasons. More recently this has been across a number of part-time roles as I have begun to combine clinical and academic roles. I am applying for the fellowship at 0.8 WTE to continue this pattern of working. I will not continue to have other NHS or charity sector roles alongside the DCAF.

4. Plain English Summary of Research

Background

Around 3,000 children and young people need treatment in hospitals for mental health difficulties each year in the UK. Children are treated in hospital when community teams cannot find a way to help and families may be finding it difficult to keep them safe at home.

When children stay on a mental health ward they have a team looking after them day and night. This team of people ensure that the whole environment around the child is helping them. The ward environment **is** the treatment – this is called a 'therapeutic milieu'.

Staying in hospital for treatment is a huge commitment for children and their families with emotional, practical and financial pressures. Given how hard it is to be in hospital, it is important for us to get treatment right.

Aims

We want to know more about how change happens when children stay in hospital for treatment for mental health difficulties. In particular, we want to know how the therapeutic milieu in hospital helps (or does not help). This will help us know how to improve the treatment process and understand when, how and why this type of treatment works. This supports children to get well enough to go home as quickly as possible, with their mental health improved.

Plans for the Project

The project will have three parts which link to each other.

Part 1

We will look at what existing studies tell us about the experiences of children and parents/carers when a child is in hospital for mental health treatment.

Part 2

All staff who work on the six mental health wards for children under 13 years old in England will be invited to complete a survey, to understand how each ward works and how they compare to each other. This will be the first time there has been a national survey of practice for children's units.

Part 3

We will use an approach to observing and understanding how things work called ethnography. This will take place at two hospitals in different parts of England. I (DC) will observe how staff work in real life to understand how the ward works at different times of day and night over many months. This will help us learn in detail the processes that staff actually take to make a therapeutic milieu.

As part of this we will interview staff, children and the parents (or carers) of the children on the wards. We will ask them about how the environment of the ward helps and does not help. Children and parents will speak to the researcher twice – once during their stay in hospital and once after they have left. We will record what they say and then summarise and interpret what they have told us.

Public and Patient Involvement

A parent told us that a member of the ward team treating their child had said to them about treatment 'we don't know how it works but it does'. This gave us the idea for the study.

Children staying on a children's mental health ward helped develop the ideas for this project and write this summary. Their parents also helped. People who stayed on children's mental health wards in hospital when they were children have also contributed. These people are all experts due to their experiences. There will continue to be a group of experts by experience who support the researcher at all stages of the project.

Sharing the Results

We will create an animation to share the results with children and parents. We will use the results to improve the way we work in inpatient mental health services nationally.

5. Scientific Abstract

How does a Therapeutic Milieu in inpatient mental health treatment enable change for children? A realist process evaluation using ethnographic methods to develop programme theory.

Background

Inpatient mental health treatment is offered to children whose needs cannot be met in any other context and whose mental health difficulties significantly impact functioning in all areas of their life. During the stay on an inpatient ward the social, emotional and physical ward environment is not just the setting in which treatment takes place; it is the treatment. Staff work to create an environment which is an effective treatment for each child. The treatment provided by this environment is described as the 'Therapeutic Milieu'.

Children accessing these services are highly vulnerable and under-served by research. Empirical research considering the working of the therapeutic milieu and in particular its application for children's mental health contexts needs further development. The complexity of the intervention and its implementation has been a barrier to research.

Aims and Methods

The aim of this research is to understand the change processes of the Therapeutic Milieu in highly specialised inpatient mental health services for children. The study is the first to consider inpatient mental health treatment for children within the framework of the UK Medical Research Council's guidance for evaluating complex interventions[1] in a theory-based approach. This project is a realist process evaluation.

Three workstreams are planned.

1. A qualitative meta-synthesis of existing studies exploring the experience of an admission for children, young people, their parents and carers.

2. A comprehensive mapping of the use of the Therapeutic Milieu in children's units in England

An electronic tailored survey and a standardised measure of ward atmosphere will be sent to all staff working in the six children's mental health units in England. The survey will explore each unit's approach to the use of the therapeutic milieu.

3. Developing an in-depth understanding of how the Therapeutic Milieu is delivered and experienced in practice

An ethnography, including participant observation and interviews with children, their parents/carers and staff from two children's units, will be completed. Interviews with each child and parent/carer will be completed at two time points; during admission and after discharge. Framework Analysis will be used.

Patient and Public Involvement (PPI)

PPI has supported development of the research questions and methods via consultation with experts by experience. Experts by experience will collaborate throughout the project with both an advisory group and specific working groups for key tasks.

Impact and Dissemination

Understanding this crucial component of inpatient treatment is central to improving care, reducing admission length and avoiding admission by identifying when it will not be helpful. Guidelines and training materials for practitioners about creating the optimal climate for therapeutic change will be shared in visits to all units and in national workshops. An animation to share the results of the study with children and families will be an impactful resource ensuring the research is accessible to all. A strong and clear foundation for future research will be established.

6. Detailed Research Plan

What is the problem being addressed?

In weekly outpatient therapy for mental health difficulties, a child spends less than 1% of their time with the therapist; but during the stay on an inpatient ward they can spend all of their time in a therapeutic environment with staff. The contribution of this therapeutic environment is described as **the 'therapeutic milieu'**[2]. The social, emotional and physical ward environment is not just the setting in which treatment takes place; in many ways, it is the treatment. Specialist practitioners work with intentionality to create an environment which is an effective treatment for each child. Understanding the workings of the milieu as the central ingredient of inpatient treatment for children is key to providing effective care and sharing best practice. However, the complexity of this intervention and the systems that provide it has presented a challenge to research in this area.

This is a crucial moment for inpatient treatment in the UK. A national review of quality in inpatient care in England was commissioned in 2019 to 'place a spotlight on services and care for some of the most vulnerable young people in our society [and] to help make these services as effective, safe and supportive as possible for thousands of families'[3]. A key aim was to ensure admissions provide effective treatment in as short a time as possible. Four key influences were identified on how children and young people experience inpatient care: quality of relationships, normality, use of restrictive practices and expectations and outcomes.[4] This project builds on the aims of the review - better understanding the central ingredient of treatment, how the therapeutic milieu 'works', would support effective treatment in as short a time as possible. The focus on the therapeutic milieu captures the influences identified as important by children and young people, particularly relationships and 'normality'.

Why is this research important?

Mental health conditions are under-served[5] in research and within this children needing inpatient care are an additionally under-served group given the even smaller amount of research for these children, whose healthcare carries such significant risks and burdens. This project improves the inclusion of this group in research.

Around 3,000 children are admitted to hospital each year in the UK because of severe mental health problems[6] which are not possible to treat in community settings, despite the active development of community services to manage more complex difficulties. Systematic reviews have demonstrated that inpatient treatment is beneficial and is effective in symptom stabilsation[7,8], but also note that there are methodological problems with existing research[9] and many studies are of poor quality[7].

There are risks and costs associated with inpatient treatment. It places significant emotional and practical demands on families and young people's lives are disrupted[10]. It is an expensive and limited resource requiring specialist staff teams[8]. It is therefore essential that when needed care is effective and high quality. To make best use of the unique opportunity provided by the inpatient environment, it is critical to have a clear and well evidenced understanding of how the therapeutic milieu enables change for children in these settings. A better understanding of how change works in a therapeutic milieu will also ensure that we can identify when this treatment is unlikely to be helpful, and admission should not be considered.

Six Children's Units in England specialize in working with children under 13 years old who need admission for their mental health. In most children of this age, it is possible to avoid admission and provide treatment at home; those requiring admission have severe difficulties which require highly specialist help. This means these units have particular expertise in inpatient treatment and provide a useful context to explore how change is facilitated by the therapeutic milieu.

How does the existing literature support this proposal?

The Therapeutic Milieu as Treatment

Much of the literature considering the contribution of the therapeutic milieu to treatment consists of clinical and theoretical papers rather than primary empirical research[11]. The therapeutic milieu approach in mental health was developed for work with adults[2]. The approach with children was more focused on residential care[12] and there is less theory and research with children in mental health contexts.

The research that does exist often only captures one aspect of the milieu setting. For example, Delaney and colleagues focus on describing the skills nursing teams use to create and sustain therapeutic milieus[13,14] noting that they require experienced staff who understand inpatient treatment to engineer them. They use four categories to delineate how a milieu is operationalized by nurses in the Four S model (safety, structure, support and self-management). Crouch also illustrates the application of the ideas in a children's psychiatric unit with case vignettes in a clinical paper[15]. The Safewards model[16] is a current key approach to creating a safe ward environment in adult inpatient mental health contexts[17]. Feeling safe is a key foundation for the creation of a therapeutic milieu but tells us less about its role in change and treatment. There is less exploration of this approach in adolescent contexts[18] and none in children's contexts.

There are qualitative explorations of adult experiences of the therapeutic milieu but little exploration in Child and Adolescent Mental Health Services (CAMHS)[19]. One study asked parents after discharge what they felt had been important in their child's treatment in general but did not focus on the milieu factors in this exploration.[20] Empirical research considering the working of milieu treatment and its application for children's mental health contexts needs further development.

Children's Experience of Inpatient Care

In terms of understanding how the therapeutic milieu may support change, including the experiences of children themselves is essential. Some qualitative research in this area has considered the experience of admissions, but there has been much less research focused specifically on how the treatment works, and almost none focused on what children and families experienced as important in the therapeutic milieu. Research on the experiences of staff, parents and young people in inpatient CAMHS settings suggests two areas (see below) that may merit further exploration.

1. Relationships in the Therapeutic Milieu

A number of studies have identified that staff, parents and young people experience young people's relationships with staff and their peers (the relational milieu of the ward) as central to inpatient treatment[21–24]. Young people felt that support to express and make sense of thoughts and feelings was important[22].

2. Involvement of Parents and Families in the Therapeutic Milieu

The child admitted to the unit has full access to the therapeutic milieu but in most inpatient CAMHS contexts the parent moves in and out of the setting (e.g. visiting at weekends). Regan[25] describes the implementation of child and family centred care on one ward. There are some units that admit a parent or carer with the child and who describe their practice in this area[26,27]. There are limited studies exploring parent's experience of their child's admission in general[24,28]. There is less exploration of how parents experience and access the therapeutic milieu or their view of how it supported change.

Research Question: How does a Therapeutic Milieu in inpatient mental health treatment enable change for children and their families?

This research will assess current practice and explore the change processes of the therapeutic milieu in highly specialised inpatient mental health services for children. This will culminate in guidelines for staff, children, and parents/carers about creating the optimal climate for therapeutic change for highly vulnerable children. To meet this aim, there are four research questions, with three corresponding workstreams:

1. What do we know about the experiences of children, young people, their parents and carers when a child is admitted for mental health treatment?
2. How is the Therapeutic Milieu currently used in children's units across England?
3. What does the creation of a Therapeutic Milieu look like in practice?
    a. How do children and families experience that this Therapeutic Milieu has helped (or not helped) during their treatment?

The research aims to be open to all children admitted to inpatient units and their families – but also notes that this is not a service to which there is equal access. Units are not evenly geographically spread and so the project includes all sites in England, and two sites rather than one for the observational studies. Structural inequalities mean minoritized groups experience barriers to accessing services and so may be under-represented in some NHS care pathways over-represented in others[29,30]. Consideration of the way inequality of access to the service may influence inequality of access to the research will be made.

Project Plan

Study Design and Methodology

The study is the first to explicitly consider inpatient mental health treatment for children within the framework of the UK Medical Research Council's guidance for evaluating complex interventions[1]. It uses a theory-based approach to understand how change is brought about in a complex system and to lead to the refinement of theory. The study is a realist process evaluation[31] to develop contextualised programme theory of the mechanisms by which the therapeutic milieu can work as an intervention in children's mental health inpatient settings.

The study uses a unique and carefully considered combination of methods to address a complex question in a complex system and to set the foundations for future research in this area, with co-design and opportunities for patient and public involvement throughout the project.

Increasing diversity in research participation was considered at planning for all stages of the research using research guidelines[5,32]. Consideration has been made of the impact of identified protected characteristics on inclusion, these will be recorded for all participants. The representation of these characteristics in the study populations will be regularly reviewed to consider whether particular groups may be excluded from participation and to address this as needed.

The three workstreams of the study are:

1. A systematic review of existing studies of the experience of an admission for inpatient mental health treatment for children, young people, their parents and carers.
2. A comprehensive mapping of the use of the therapeutic milieu in inpatient treatment in children's units in England via a national survey
3. Developing an in-depth understanding using ethnography of how the Therapeutic Milieu is delivered and experienced in practice in inpatient child mental health treatment

The study is predominantly qualitative in methodology, using multiple methods. The quantitative element is small but consideration of the key decisions in planning mixed methods research as below has supported the planning of these multiple methods[33] ;

- Interaction: There will be interactions between workstreams; the theory and results will build in successive workstreams.
- Priority: Qualitative methods are emphasized with the quantitative element taking a secondary role.
- Timing: Elements are planned sequentially, initial analysis of earlier workstreams supports the planning of later data collection.
- Mixing: The model for analysis will support mixing of results from different workstreams at the later analysis stages.

1. A systematic review of existing studies of the experience of an admission for inpatient mental health treatment for children, young people, their parents and carers.

I have begun a systematic scoping review using JBI methodology[34] to assess the scope of literature relating to the use of therapeutic milieus in inpatient mental health treatment for children and adolescents and to understand the ways in which the therapeutic milieu concept is described and applied in this literature. This will review be completed in the second half of my pre-doctoral fellowship this year and will be a key foundation for the doctoral study. As per this methodology the Population, Concept and Context criteria are specified below[35]

- Population: Children and Young People (under 18 years) with complex mental health difficulties
- Concept: Therapeutic Milieu
- Context: Specialist Inpatient or Residential Mental Health Unit or Hospital

During the doctoral fellowship, building on the scoping review, a systematic review of the experiences of children, young people and parents where a child or young person is admitted for inpatient mental health treatment will be completed. This will include qualitative literature and explore key areas that staff, children and families identify as important in treatment.

There are some systematic reviews of outcome in inpatient CAMHS[7,36] but very few considering experience. Existing reviews of qualitative literature on experience exclude people under 18 years old[37] or focus on forensic

services[38]. There is one qualitative meta-synthesis of adolescent experiences but it is notable that useful qualitative studies referenced here[21,23,24] were not identified in its search strategy indicating it would benefit from a review and update. The recent clinical review of services in the UK[4] included qualitative literature on young people's experiences but was a clinical review rather than a systematic one.

Method

A qualitative meta-synthesis[39,40] will be completed following good practice guidance around process and reporting.

2. A comprehensive mapping via a national survey of the use of the therapeutic milieu in inpatient treatment in children's units in England

There is no existing mapping of the approaches used by different mental health units to treatment in the UK and limited clear guidance as to evidence-based practice. There is also little description of this internationally and inconsistency in the way that it is described[41]. This workstream will produce essential practice-based evidence by mapping current practice in the UK and exploring how ward environment varies across units.

Sample

All professionals working on children's inpatient units in the UK will be invited to take part. Recruitment will be supported by visits to all children's units and a key identified contact at each unit. Participants will be offered a gift voucher as a thank you for completing the questionnaire to maximise the response rate. The representation of different staff groups and demographics in the responses will be reviewed and an iterative recruitment strategy used so that the distribution plan can be adjusted to include any missing voices and groups. To prevent digital exclusion, the survey will be accessible on a smartphone and access to the internet for the survey purposes provided at each workplace[32].

Benchmarking in UK children's units suggests that around 250 staff work across six children's units in the UK, the majority as nurses or healthcare assistants[42]. The annual national NHS staff survey has a 50% response rate and no incentive is offered for completion – this is used as an indicative response rate. A 50% response rate would give a sample of around 120 staff.

Method

An online tailored survey will be developed in consultation with staff from the units and the steering group. It will cover key topics exploring the unit's approach and how this considers the use of the therapeutic milieu. The survey will be designed, piloted and tested for validity.

Whist standardised measures of ward environment exist these have rarely been used to consider how the environment may vary across units. This study will use these for the first time in children's units. The Ward Atmosphere Scale[43] has 100 true or false questions in full version or 40 items in shortened version, client and staff and real and ideal versions. It is widely used in clinical settings and program evaluation[44].

Analysis

Anonymous data from the survey and Ward Atmosphere Scale will be transferred to excel and descriptive statistics will be used in the analysis, grouping responses by unit and profession as appropriate. Relevant qualitative data from more open questions will be thematically analysed.

3. Develop an in-depth understanding of how the Therapeutic Milieu is delivered and experienced in practice in inpatient child mental health treatment

Ethnography will be used as a central approach to enable direct observation of the use of the therapeutic milieu in treatment. Ethnography is concerned with culture, shared practices and beliefs, and how the social context shapes, and is shaped by, individuals[45]. It is increasingly being used as a method in health contexts[46], including on adult inpatient mental health units[47,48]. This methodology uniquely enables the research to capture the complexity of the intervention and the interactions that support it in a setting. Ethnography bridges the gap between how people might describe what they do, as captured in the survey, and what they actually think and do. Fitting with the MRC complex intervention framework, this will produce results relevant to real world context in the NHS.

Sample

Ethnography will be completed on two sites sequentially. Qualitative health researchers describe that 'the most insightful and useful data came from those sites we were able to study in the greatest depth over the longest time'[49]. Understanding the particularities of a small number of contexts in detail enables the creation of theory and concepts which are useful universally – rather than aiming for a statistically representative sample the sample aims to choose settings from which we can learn the most[50].

One site, the Mildred Creak Unit (MCU) at Great Ormond Street Hospital has used the therapeutic milieu as a cornerstone of its approach over many decades, as previously described by clinicians who have worked on the unit.[15,51] The second site will be chosen from other children's units to explore how similar ideas are used in a different geographical and service context.

The later phases of the ethnography on each site will include semi-structured interviews with children, families and professionals.

- For staff, purposeful sampling will ensure the selection of key informants considers the balance of professions working on unit and the demographics of the staff team. Given typical numbers of professionals on a ward, 8-10 formal interviews across both sites are estimated to be needed alongside the informal ones which accompany the observations.
- For children and parents/carers, it is estimated that 10-12 children and parents will be interviewed across the two sites. This number is based on sample sizes for previous qualitative studies in equivalent settings[20–22]. Inclusion criteria are current admission to a children's inpatient unit for treatment at the point of recruitment, there are no exclusion criteria. Purposeful sampling will be used recruit a sample including children with different diagnoses, demographics and family structures.

For children and families, any expenses incurred associated with participation will be covered. Given that I speak only English fluently, where a different language is preferred by the participant 'real-time' translation will be used. The researcher's familiarity from their presence in the ethnography at these sites will support the participation of a vulnerable client group where trust will be important to enable engagement in research[52].

Methods

Participant observation involves the researcher spending time with the people and context of the ward to allow an everyday experience of treatment. The duration of observation has been estimated using reference to published ethnographic studies and in consultation with an academic supervisor (CV) experienced in these methods. The visits to units early in the project will support initial ethnographic orientation to the field and I will record my observations and reflections.

Ethnography on the first site will be completed over a period of six to nine months with data collection over three to four months. Ethnography at the second site will be completed over three to four months allowing a period of two to three months for observation. The observation at this site will be completed after that on the first site and the reduced time at this site reflects the data already collected.

The observations will be purposefully planned to observe the unit at different times of the day and week and to include key ward contexts such as ward rounds, mealtimes and handovers. Consultation with the advisory group will consider how to make the observations the most comfortable for children. The developing programme theory (informed by the literature reviews, the orientation to the field in the visits to units and the survey results) will also support the planning of observations. Informal interviews will occur alongside the observations as the researcher asks people on the ward about the observations as they happen. A field diary will be used to record observations.

After an initial phase of observation and informal interviews, more formal semi-structured interviews will take place to support the development of the data and provide triangulation. The interview guide will be informed by the developing analysis of the observations and with the support of the advisory group. Interviews can be conducted in person or remotely. Where helpful, expert by experience co-researchers may support interviews. Creative methods will be used as helpful with children and the adult interviews will also be multi-modal to support communication i.e. not just rely on verbal questions. Interviews will be audio-recorded and transcribed and written materials scanned.

For children and parents, each will be interviewed at two time points, one during the latter stages of admission and one after discharge. This is to help them describe their experience of change and support reflection on the experience of inpatient treatment.

The observations and interviews will continue to the point where the information power of the sample is considered

adequate[53].

A reflective research diary will be used to support reflexivity. In the reflective diary, particular consideration will be given to my own culture and biases to support the ongoing development of cultural sensitivity and understand their influence on the observations. There will be an intentional break between my work as a clinician on MCU ending and me rejoining the context as an observer to allow a transition in roles. Supervision will support me in consider how I position myself and utilize my insider perspective to the advantage of the project.

Analysis

Analysis begins in parallel with data collection because data collection will adapt to explore gaps or salient topics. Framework Analysis[54] will be used as an approach. It enables pre-existing knowledge to contribute to analysis but also enables new ideas and understanding to emerge. There will be iterative stages of analysis of the emerging dataset so that the early analysis can guide later data collection. My supervisory team has experience in applying this approach in a psychology context[55] and with qualitative research[56].

As initial themes and interpretations emerge, these will be shared with experts by experience and with professionals on the unit to support their iterative development and develop engagement with the research.

Outputs and Impact

Planned outputs connect to all key stakeholders to ensure maximum impact:

1. A co-produced animation/s will enable families, young people and children to understand how treatment as an inpatient will help – this will improve both the experience of treatment and also the effectiveness of treatment as it supports shared goals and understanding. It will ensure the research has an ongoing impact as it will be shared with children and families for many years to come. Posters and infographics will be created for display on wards where children, parents and staff can access these.
2. For the first time, practitioners will have clear evidence-based guidelines to enable the creation of a therapeutic milieu and the optimal climate for therapeutic change
3. Clear programme theory and a logic model for the use of the therapeutic milieu as an intervention will support future research, policy development and service development nationally
4. Four submissions will be made to peer reviewed journals; the systematic review, practice survey, ethnography and interviews
5. Conference presentations and workshops will share the learning nationally and internationally in both mental health contexts and academic contexts where the results will enable the development of psychotherapeutic theory

The research will ensure that when vulnerable children and young people need inpatient treatment for mental health difficulties they are able to access high quality and effective care. This research will enable all units in the UK to deliver more focused and effective care, supporting shorter admissions and also avoiding admissions which are likely to be ineffective. This will enable significant efficiency savings to be made.

The project will have impact on a national and international scale during the doctoral study due to the focus on engagement and collaboration. It will engage stakeholders and activate research in an under-studied area where it is much needed, building networks for ongoing research among units.

Engagement and Dissemination

In the early months of the project visits will be made to all six children's units in England to support engagement with the project, participation in the research and future dissemination of the results. Visits will be repeated in the later stages of the project to allow triangulation of results and dissemination and ensure impact. A workshop will be run as part of the Quality Network for Inpatient CAMHS annual forum to maximize attendance and impact. The learning will be shared internationally on the overseas visit and at international conferences.

The animation will be produced with experts by experience and launched in contexts where they feel it will be most impactful. The launch will be supported by the GOSH media team via the hospital social media and website.

Links with NHS England and policy makers via my current clinical role and my academic supervisory teams will be utilized to contribute to relevant guidelines and service development.

Learning about the ways in which environments can best offer treatment and support for mental health difficulties

is highly generalizable to many contexts, as diverse as schools, residential care settings and prisons. Sharing results in both mental health and academic contexts will enable this generalization. The study will develop the theories of change in psychotherapeutic literature by exploring their application in a context where the therapeutic relationship is between a system and a young person rather than a dyadic one with a single therapist.

Future Research

This project addresses the core elements needed as part of the research framework to evaluate complex interventions[1] and builds an excellent foundation for the development of an active future research programme considering inpatient treatment for children. It will;

1. Develop a Programme Theory (and Logic Model) for mental health treatment using a therapeutic milieu
2. Engage stakeholders with the research topic and build networks for future research
3. Identify the key uncertainties via review of the literature and by understanding the experience of staff, children and families using a qualitative exploratory model

An application for further funding would be made, either as a project in itself (e.g. NIHR Research for Patient Benefit) or as a further fellowship. There are bridging schemes accessible to post-doctoral fellows provided by GOSH BRC which will support me in developing these applications.

Project Management

I will access learning about PhD management and will also apply the project management skills I have developed in my clinical roles and pre-doctoral fellowship. My supervisory team is well experienced in supporting PhDs and we have established an effective model of supervision. My practice supervisors are familiar with the clinical and research context at GOSH and so will also provide additional support. Alongside my supervisory team, a thesis committee is part of the PhD structure at UCL to ensure the project remains on track. I will meet at least twice a year with the GOSH finance manager to support the recording of expenditure and to facilitate reports to NIHR.

I am proposing a fellowship at 0.8 WTE because I have worked at 0.8 WTE over many years for personal reasons. However, the extended duration of the fellowship to 45 months will also allow time for NHS ethics processes without compromising time for data collection or write up. There are significant advantages to a longer window for recruitment given the relatively small numbers of children admitted each year.

Ethics

The study involves no changes to routine NHS care and is observational. A review of Health Research Authority guidance and comparison to similar projects would indicate that the survey of professionals in workstream 2 will need R&D approval but does not need NHS REC ethical approval - Ethical approval will be sought from UCL REC. NHS REC ethical approval will be required for workstream 3, and this will be applied for, aiming for approval within the first six to nine months.

The researcher will be present on wards during emotional and stressful times when children, families and staff feel vulnerable. Obtaining informed consent in the power dynamic context of inpatient care will need careful consideration. A protocol for support with distress and advice for how to access support will be developed, this will include consideration of the impact of the research on the members of the expert by experience advisory group. Safeguarding procedures will be carefully considered as part of the study protocols.

Data will be pseudonymized and handled in accordance with relevant data protection legislation, consideration of secure storage has been made and the service identified for transcription is approved by GOSH as compliant with data protection responsibilities. Information governance procedures will be followed.

Success Criteria

Key measures of success for each workstream and the plans to manage the risks associated with these are specified below.

Workstream 2

Success criteria: 40% survey response rate, fully completed surveys returned, representative sample

Risk management: focus on engagement in project, pilot survey, voucher incentive, identifying key individuals at each site who can support additional responses as needed

Workstream 3

Success criteria: Completion of hours as planned on site for ethnography and adequate numbers of individuals for interviews as per estimates

Risk management: contingency plans to complete on another site (or to reduce to one site), wide recruitment window, protocol could be adjusted to contact previous patients for an interview after discharge and significantly widen the pool of potential participants

PPI

Success criteria: Establishing a well-attended and engaged advisory group

Risk management: some members already identified, further development over the next year prior to the PhD starting in September 2025

Adoption and Implementation:

Success criteria: results shared with majority of children's units and at 2 national conferences attended by clinicians

Risk management: changes to services nationwide implemented by commissioning bodies or significant shifts in the emphasis on inpatient care could impact adoption - engagement with NHS England and national networks will be important to manage this risk.

7. Patient & Public Involvement

Please describe how patients/service users, carers and the public have been involved in developing this proposal

Patient and public involvement (PPI) is a key focus of my bridge fellowship. A consultation approach[57] supported the development of the research focus, funding application and methods. Payment was offered as per NIHR and NHS trust guidelines.

Involvement to date:

1. March - May 2024: Six young people currently admitted to an inpatient unit helped develop the research questions and focus and reviewed sections of the Plain English summary.
2. Previous patients and families who had expressed interest in involvement in research were asked if they would like to be involved in developing the project. Two parents and two young adults (previous patients) took part in online video consultations to consult on the research question and plain English summary.
3. In May 2024 the Plain English Summary was shared with these experts by experience (current inpatients, previous patients and parents) to request feedback both on the clarity of the summary and the project plan.
4. A member of the GOSH young person's advisory group (YPAG) offered consultation to support the development of the project.

Focus groups had been planned but on exploration with the individuals involved some preferred to meet 1:1 and differing ages made groups hard to form.

Individuals expressed that research in this area felt important because they hoped it would support continued access to inpatient treatment for children. Others felt the research would support a needed process of improvement in inpatient services. One felt that it was important to give voice these experiences as so few people understand them. A parent noted that the outcome should not be 'a piece of paper no one reads'.

Sixty-three exit interviews completed over the last five years by families on discharge from MCU were audited[58] to identify any themes that connected to possible research questions. There were themes related to the impact of the milieu on children's treatment (structure, peer group) in both positive and negative ways. This supports the idea that this should be better understood.

Published research priorities were referenced, identifying that research which addresses the needs of children with complex intersecting needs was considered important[59].

Please describe the ways in which patients/service users, carers and the public will be actively involved in the proposed research, including any training and support provided

The study will be a collaborative project with co-designed elements. Involvement from key experts by experience will support the research project at all stages including:

- the design process
- data collection
- data analysis and interpretation
- the sharing of the results at conferences and via other mediums

The invite to involvement will be advertised widely to include different perspectives and widen the voices included in the project. I will advertise via the national network of children's units and run events such as open days to engage previous patients. Individual meetings will identify any support needs and a person's hopes for involvement.

This high level of involvement reflects the intention of the research project to ask questions which focus on the perspectives of children and families and to develop a research project exploring sensitive topics in a respectful way, addressing the power imbalance in research relationships.

GRIPP2 checklists[60] will be used to support planning and reporting. In the early stages of the project the options for involvement will be further developed as the advisory group forms and individuals can consider which best fits for them. They are likely to include:

1. UCL Thesis Committee
2. Lived Experience Research Advisory Group - Initial PPI suggested that experts by experience would prefer a separate group rather than a group with professionals.

3. Specific working groups to support key tasks such as
    a. reviewing information sheets
    b. developing interview schedule
    c. develop animation or story books to share findings.
4. Co-researcher e.g. joining interviews to support voice of young person or raise things they think the young person might be uncomfortable to raise. If individuals are interested in joining as co-researchers appropriate training will be offered (e.g. in research interviews or ethics).

Consideration of supporting neurodiverse needs will be made in all contexts. If diverse voices are not represented in the PPI network then the advertising and invite strategies will be reviewed to address this. If interpreters are needed to support involvement this will be provided. A payment policy for the project will support appropriate payment following NIHR and local guidelines. Provision has also been made for training, childcare and carer payments.

If it is considered not appropriate and meaningful to actively involve patients/service users, carers and the public in your proposed research, please justify why
-

8. Training & Development and Research Support

Proposed training and development programme

Proposed Formal Academic Study

The Doctoral Skills Development Programme at UCL provides access to varied taught and online learning with doctoral researchers spending 10 days a year on skills development. Key elements needed to develop as a researcher and academic leader have been planned using the VITAE researcher development framework, the elements planned are listed below (listed sequentially within each domain). These will be planned across the 45 months of the fellowship.

Domain A: Knowledge and intellectual abilities

- Data Collection and Analysis: Software Success: data analysis, visualisation, survey tools
- Principles and Tools for Literature Review: literature searching, quality appraisal of qualitative research
- Qualitative Methods and Analysis: thematic analysis
- Tools for Research Communication: multimedia content

Domain B: Personal effectiveness

- PhD journey workshops: starting, reading, producing thesis and reports
- Researcher's skills: leadership
- Academic careers planning for PhDs

Domain C: Research organisation and governance

- Writing data management plans
- Research ethics and integrity
- Introduction to generating grant funding
- Persuasive grant writing
- Research project management

Domain D: Engagement, influence and impact

- Making the most of your supervisory relationship
- Paper Writing and Thesis Writing Series
- Presentation and pitching skills
- Engagement and Impact: skills workshops
- Developing as a PGT supervisor

Additional Academic Study

During my PCAF bridge I have been able to build a foundation of research skills relevant to this project. I have completed training in scoping review methodology, an introduction to ethnography in health research and training in complex intervention research run by the team that developed the guidelines for this.

The Doctoral Skills Development Programme at UCL is comprehensive and will support many of the required elements of my academic development. Alongside this additional study will support the particular research skills needed for my planned project. There will be a focus on learning these methodologies in the early years of the PhD so that I am able to plan and utilise the methods appropriately.

Year 1:

- Essentials of Survey Research Design (Royal Statistical Society)
  This will support the design of the survey of practice
- Rapid Research and Evaluation Course (RREAL Rapid Research Evaluation and Appraisal Lab, UCL)

This will develop my understanding of the use of ethnography as applied in health contexts.

- Qualitative and Mixed Method Systematic Reviews (University of Oxford)
  Building on my training in scoping reviews, this will develop my skills in qualitative reviews, relevant both to my PhD and important in my career as a researcher.
- Inpatient CAMHS network annual forum (Quality Network for Inpatient CAMHS)
  Attending this forum of workshops and presentations supports both the development of subject knowledge and ensures I will stay connected to clinical practice. It will support engagement and dissemination of the project

Year 2:

- Qualitative Analysis course (National Centre for Social Research)
  This course is focused on 'Framework', an approach to qualitative data management developed by this centre and which will be used throughout the qualitative analysis of the study

Proposed Practice Development

Developing specialist practice as a senior clinical psychologist includes developing both advanced therapeutic skills and expertise in applying these in a particular context. As a senior clinician, my planned practice development also includes training to support my development as a leader, manager and supervisor.

Advanced Therapeutic Skills: Mentalisation Based Therapies (MBT)

Developing specialist therapeutic skills during my fellowship will enable a future clinical-academic career with a focussed and relevant specialism. This training will be in the latter years of the DCAF to allow a focus on methodology and research skills in the earlier stages. Training will enable me to develop further specialist expertise in Mentalisation Based Therapies. As a psychologist, this is a key evidence-based approach applicable both to practice in the therapeutic milieu and to individual therapy. Both my supervisors have roles at Anna Freud - a world leading mental health charity and centre of excellence in MBT.

The MBT training will support development both as a practitioner and as a supervisor in this approach, which fits with my clinical role as a senior practitioner with responsibility for supervising others. I have previously completed the introductory and adolescent modules of this programme. During the fellowship I will complete the child and family modules and join an ongoing specialist practice group to develop my skills. When attending the practice development group, my practice supervisor will support me in facilitating the small amount of supernumerary clinical work I need to achieve certification in this approach.

I will also audit or attend training in AMBIT (Adaptive Mentalization Based Integrative Treatment). This applies the ideas of mentalisation to systems of help around particularly vulnerable, excluded, and underserved clients. This will help me connect the model I use in individual therapy to work at a system level on inpatient wards.

The visits to Children's Units across England at the start and end of my PhD will develop my expertise in inpatient care and provide a unique opportunity to develop specialist practice. I will also attend the annual forum of the inpatient CAMHS network each year as this forum will connect me to current practice and policy development in inpatient CAMHS in the UK.

Leadership and Management:

In the last two years of the PhD I will attend a leadership training to support me in developing my leadership skills and in translating my leadership skills from an NHS context to an academic one. Tavistock Consulting have an approach to leadership which fits with both NHS contexts and the ways in which my own practice has developed and will continue to develop. I will also attend training in clinical supervision to support me in my role as a senior clinician supervising others.

Primary Doctoral Supervisor

PRIMARY DOCTORAL SUPERVISOR 1

Name	Professor Julian Edbrooke-Childs

Institution	University College London

Position	Professor of Evidence Based Child and Adolescent Mental Health

Description of support provided
I am Professor of Evidence-Based Child and Adolescent Mental Health at UCL and Co-Director of the Evidence Based Practice Unit (EBPU), an applied child and adolescent mental health research group across UCL and the Anna Freud Centre. I have secured over £15 million in research funding. I have utilised multiple methodologies both qualitative and quantitative in addition to systematic reviews. I have authored over 100 peer-reviewed publications. Relevant in supervising Dawn is research with vulnerable populations of children and young people. I lead two national evaluations in the children and young people's secure estate - mixed-methods realist process evaluation designs. I am co-PI on a project examining the implementation of a new role in inpatient mental health settings. I have a track record of involving experts by experience in the design and delivery of research. My extensive experience of policy research will help Dawn to situate her research within the current policy landscape for children and young people's mental health and devise concrete plans to ensure the wider impact. I am the Module Lead at UCL for a course on applied child mental health research for doctoral and MSc students and I am the Programme Coordinator for a UCL PhD programme on evidence-based child and adolescent mental health. I have four current doctoral students and have supervised six completed doctoral students. I have supervised Dawn fortnightly throughout her PCAF and will continue this for the DCAF. Dawn will also join lab meetings with other PhD candidates.

Additional Academic Supervisor(s)

ADDITIONAL ACADEMIC SUPERVISOR 1

Name	Professor Nick Midgley

Institution	Anna Freud National Centre for Children and Families

Position	Co-Director of the Child Attachment and Psychological Therapies Research Unit (ChAPTRe)

Description of support provided
Professor Nick Midgley is Professor of Psychological Therapies for Children and Young People in the Research Department of Clinical, Educational, and Health Psychology at UCL and Anna Freud. Professor Midgley's primary research interest is in psychological therapies with children and young people, with a particular focus the use of qualitative and mixed methods. Professor Midgley is currently chief investigator on the National Institute for Health Research (NIHR) funded Reflective Fostering Study as well as the Emotional Regulation in Children clinical trial, evaluating mentalization based treatment (MBT) for school-age children with emotional and behavioural problems referred to child and adolescent mental health services in the UK. He has

secured over £9m of funding for research and will be able to support Dawn to consider future funding sources to continue her post-doctoral research.

Professor Midgley is the Academic Course Director for the professional doctorate in Child and Adolescent Psychotherapy (DPsych). This means he has particular expertise in supporting NHS clinicians with research and in the skills needed for research in the context of the NHS.

He has almost 200 academic publications and has supervised 28 PhD and PsychD students at UCL. To date, 22 of these have been supervised to completion, a number remain in progress and there will be opportunities for Dawn to link to current PhD students.

Professor Midgley will be available for one meeting per month as a secondary supervisor, supervision will be provided jointly with Professor Edbrooke-Childs. This has worked well during the current fellowship as effective supervision.

ADDITIONAL ACADEMIC SUPERVISOR 2

Name	Professor Cecilia Vindrola

Institution	UCL

Position	Director of the Rapid Research Evaluation and Appraisal Lab

Description of support provided
Professor Cecilia Vindrola is the Director of the Rapid Research Evaluation and Appraisal Lab (RREAL) and a Professorial Research Fellow in the Department of Targeted Intervention at UCL. She has supervised 5 PhD students to completion, with an additional 2 students completing this year. Prof Vindrola is a medical anthropologist, trained formally in ethnographic research approaches. Over the past 10 years, she has worked with other social scientists to develop new approaches for rapid and collaborative ethnography due to her interest in applied health research. She has written extensively on this topic, including the book titled *Rapid Ethnographies: A Practical Guide* (Cambridge University Press). For the purpose of this project, she will provide specialist methodological supervision on the use of ethnographic research and the implementation of fieldwork in NHS settings.

Clinical Supervisor(s)

PRACTICE SUPERVISOR 1

Name	Dr Tara Murphy

Institution	Great Ormond Street Hospital

Position	Consultant Clinical Psychologist

Description of support provided
Dr Tara Murphy is a Consultant Clinical Psychologist who has worked at Great Ormond Street Hospital since 2003 and is part of the senior management team in the Psychological and Mental Health Services department. She holds an honorary academic role as a Consultant Clinical Psychologist with the University of Nottingham and has held other academic roles. Dr Murphy has co-authored over sixty publications and has written four books. Her books communicate psychological research and ideas in ways that are accessible to both professionals and the public.

She has successfully combined development and progression in her clinical role with integrated academic roles and research, using different models to support the production of these paths in parallel. She has been a principal investigator for successfully funded applications, including from the NIHR (Research for Patient Benefit; Health Technology Assessment Programme and i4i grants).

Dr Murphy will be able to support me in developing as a professional in a clinical-academic role during the fellowship and in planning a future career as a clinical-academic. She will help me to develop both as a senior psychologist and as a researcher in the context of my department and the hospital.

My clinical psychology doctoral thesis in 2006-2007 was supervised by Dr Murphy and we published the research. As such, I am confident in our ability to establish an effective supervisory relationship. Dr Murphy will meet me termly and support both practice and professional development, we can adjust our meetings during different phases in the fellowship.

PRACTICE SUPERVISOR 2

Name	Dr Sinead Marriott

Institution	Great Ormond Street Hospital

Position	Consultant Clinical Psychologist

Description of support provided

Dr Sinead Marriott is a Consultant Clinical Psychologist who is a member of the Senior Management Team in the Psychological and Mental Health Services (PAMHS) department at GOSH. She leads the management stream which includes the psychologists working on the Mildred Creak Unit and so is familiar with the inpatient CAMHS context.

Dr Marriott is an experienced supervisor and manager and has supervised and managed me for the last three years. She has been my line manager and supervisor throughout my pre-doctoral bridge fellowship. She has helped me structure my concurrent part-time research and clinical roles so that both are well boundaried.

Dr Marriott will hold my line management in the department during the fellowship. Dr Marriott will identify appropriate pieces of therapy work to support my professional practice development in mentalisation based therapies as part of my training and development plan. Her role as team lead will mean that she is well placed to identify suitable children or families.

Collaborations

UK Inpatient CAMHS Clinical and Research Networks

A key collaboration is enabled by membership of the Forum for Inpatient Child and Adolescent Psychology Services (FICAPS), a national network within the British Psychological Society. During the PCAF bridge I have set up a specific smaller network within this for psychologists working in children's inpatient units. The research project has been discussed in both of these forums and this has built engagement with the project.

Visits to all children's units in England form a key element of the plan to support engagement with the project and develop further collaborations. The learning from visiting these contexts will also provide exceptional experience to enable my development as a clinical-academic specialising in the area of inpatient treatment. The Paediatric Excellence Initiative, a current partnership between the GOSH BRC, Alder Hey Children's Hospital, Birmingham Women's and Children's Hospital and Sheffield Children's Hospitals will also facilitate links with the children's units at Alder Hey and Sheffield and provide a shared supportive research infrastructure.

In preparing this study proposal I have explored the UK research landscape to identify active researchers working in the area of inpatient CAMHS – it has been notable that I have not been able to identify an active research group specialising in this area, although a number of individuals working in related areas have been very helpful. For example, I have met researchers based at the Mental Health Policy Research Unit who have recently reviewed approaches to inpatient care[61] and connected to previous NIHR fellows who have completed research in inpatient CAMHS services[21]. I will continue to build my own network.

International Inpatient CAMHS Clinical and Research Contexts

During the Bridge award I have made connections with individuals identified as leading research internationally in the inpatient CAMHS context. I have also had the opportunity to discuss the project with clinician-researchers active in the international psychotherapeutic context via my current supervisors.

Professor Kathleen Delaney is an emeritus professor in the US, linked to Rush University in Chicago, who studied inpatient psychiatric treatment environments over the course of her career. She has offered help as needed and supported contacts to other units internationally who are centres of excellence in working to develop the therapeutic milieu. One such unit is Banksia Ward, an adolescent unit in Melbourne Children's Hospital that has worked across the last 12 years to establish a therapeutic milieu. I have met with the ward team online and we will continue to develop our connections and collaboration during my doctoral fellowship.

Dr Clare Hayes is a researcher at Monash University in Melbourne who completed her PhD on adolescent mental health units and models of care and continues to work as a researcher in this area. We have discussed the development of this application and she has agreed to be involved in the steering group for the project. Dr Hayes will also support further connection to Orygen youth mental health research network in Melbourne.

A visit will be planned to connect to these researchers in Australia active in this area and to visit the wards and hospitals identified above as centres of excellence in this area.

Host Organisation Support Statement
The following statement of support should be composed jointly by the Heads of Department / Senior Managers of the employing host organisation and the partner organisation.
The Heads of Department / Senior Managers of these organisations will be able to add the statement of support to the form once they have confirmed participation. Invitations to participate are sent by the applicant via the 'Participants and Signatories' section of the form.

Statement of support

Great Ormond Street Hospital (GOSH):

Great Ormond Street Hospital (GOSH) is a Research Hospital where research is an integral part of the working lives of our staff and the patients and families we treat and see. GOSH and UCL Great Ormond Street Institute of Child Health (ICH) were awarded NIHR Biomedical Research Centre (BRC) status in 2007, the award was renewed at every term end, most recently in 2022. We are the only solely paediatric-focussed BRC in the UK. There are strong support structures hospital wide for research which will benefit Dawn in her fellowship and in continuing her practitioner-academic career after her fellowship. For example, our participation lead can support Dawn with development of PPI in the GOSH child health context.

The Centre for Outcomes and Experience Research in Children's Health Illness and Disability (ORCHID) will continue to support Dawn as an early career researcher. Since 2016 the ORCHID team has supported 33 AHPs and nurses in being successfully awarded NIHR clinical-academic fellowships, 17 of these were doctoral fellowships. Eight of these doctoral fellows have completed their funding and successfully defended their thesis, three of this group now have post-doctoral awards from the NIHR including one advanced clinical academic fellowship. Partnership with UCL for NIHR fellowships is well established with all of the above non-medical fellowships having been partnerships between GOSH and UCL.

Dawn works in Psychological and Mental Health Services (PAMHS). Leading in research is a top five priority in the department's strategy and there are clear research structures, for example the research committee which Dawn is a member of and the research support group which Dawn has consulted on the development of her proposal. Dawn has been the first psychologist in the department to be awarded an NIHR fellowship and we are now using the same pathway to support other psychologists to access these fellowships, learning from successful

models of support for early career researchers in other allied health professions at GOSH.

Dawn will be supported to leave her her clinical role Mildred Creak Unit (MCU) to move to a research role. The strong relationships with the MCU team built over many years in her clinical role will support the research project. We will support her clinical development and training programme by enabling the discrete pieces of therapy work she will need for her mentalisation based therapy training. She will be in a supernumerary role and will only complete the clinical work needed for continued registration and her training and development plan.

During the fellowship Dawn will have review meetings annually alongside routine six-weekly line management which will support her and the department in shaping a post-fellowship role. We have a number of senior research focussed psychologists linked to the department who will be able to guide this, one of these, Dr Tara Murphy, is a Practice Supervisor for Dawn.

GOSH has a strong diversity and inclusion framework 'Being seen and being heard' and a commitment to addressing systemic issues and ensuring that everyone feels able to bring their whole self to GOSH. GOSH is a Disability Confident Employer and a Stonewall Diversity Champion with active Race, Ethnicity and Cultural Heritage (REACH), The Pride Network, Enhancing Abilities and Leveraging Disabilities (ENABLED) and Women's staff forums.

UCL:

UCL is rated second in the UK for research power in the last Research Excellence Framework (2021) and the top-ranked university in the UK for research in Psychology, Psychiatry, and Neuroscience. The Division of Psychology and Language Sciences (PALS) is world-leading in research and teaching on mind, behaviour, and language.

Dawn will be located in the Evidence Based Practice Unit (EBPU). Founded in 2006, the EBPU is a collaboration between UCL Faculty of Brain Sciences and Anna Freud. It bridges cutting-edge research and innovative practice in children's mental health, and Professor Julian Edbrooke-Childs co-leads this group. Dawn's project will be well supported by the unit's expertise in mental health research.

Dawn will also be connected via Professor Nick Midgely to the Child Attachment and Psychological Therapies Research Unit (ChAPTRe), another UCL-Anna Freud research group. ChAPTRe has a focus on mechanisms and models of change in psychotherapy, which will support the project's exploration of the way that the theoretical approach of an inpatient unit influences treatment.

Being situated with these research groups will enable Dawn to access infrastructure, support, and learning across multiple groups and organisations. There is a seminar series on Children and Young People's Mental Health, methods-specific peer support workshops, and lab meetings for PhD students. Dawn will have access to other resources, including library services, which provide support to researchers when conducting literature reviews, and a range of courses and learning opportunities. Other development schemes will be available, such as mentoring and shadowing of projects and areas of the organisations for practice-based learning. UCL provides research integrity training, and lab meetings provide ongoing spaces to discuss ethical and safeguarding practices in the context of research.

UCL is committed to the aims and principles of the Race Equality Charter and was one of the first institutions in the UK to be formally awarded for its efforts to understand and take steps to address racialised inequalities. UCL received a Bronze award in 2015 and this was renewed in 2020. UCL's equity, diversity, and inclusion (EDI) strategy hardwires EDI into UCL's culture, teaching, and research. In addition to the top-down EDI initiatives, Dawn will have access to staff-led working groups which provide training and events, safe spaces for reflection, and opportunities to participate in organisation change through policies and practices.

UCL has been instrumental in the advancement of the Concordat to Support the Career Development of Researchers and is a proud signatory of the revised Concordat, published in 2019, which commits institutions to driving forward researcher development. The UCL Research Strategy sets out our ambition to nurture, develop and celebrate those engaged with research.

9. Detailed Budget

The finance section should provide a breakdown of costs associated with undertaking the research as described in the proposal.

1

Staff type	Lead Applicant

Name	Dawn Cutler

Employing organisation	Great Ormond Street Hospital For Children NHS Foundation Trust

Grade	8B

Starting point on scale	Top

Increment Date	02/11/2021

Justification of Costs
I am already at the top of my pay band and so increment points do not apply.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	59580
Geographical Weighting (£)	6197
Other allowances (£)	0
Superannuation (£)	15642
Nat. Insurance (£)	8366
Current Annual Costs (£)	**89785**

Direct Costs
These are the costs that are specific to the research, which will be charged as the amount actually spent and can be supported by an audit record.

Staff Type: (Lead Applicant)-Dr Dawn Cutler
Type of cost: NHS
% Full-time on this research: 80.0
Total months on this research: 45.0
Justification of Cost: I work part-time for personal reasons and will continue to work 4 days / week throughout the fellowship, which takes it's duration to 45 months.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	59580	59580	59580	44685	223425
National Insurance	8366	8366	8366	6274	31372
Superannuation	15642	15642	15642	11731	58657
Geographical weighting / Other allowances (if applicable)	6197	6197	6197	4648	23239

Totals	Applicant	Support Staff	Shared Staff	Total Cost (£)
Total HEI	0	0	0	0
Total NHS	336693	0	0	336693
Total Commercial	0	0	0	0
Total Other	0	0	0	0

Travel, Subsistence and Conference Fees

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Travel and Subsistence for Advisory Group and PPI Activities
Justification of Cost: Includes an estimate of travel costs and subsistence to support attendance at: 1. Advisory group: 3 times/year for 6 people 2. Expert by experience to join thesis committee: 3 times/year for 1 person 3. Working groups: 5 people per group to support workshop activities such as such as designing interview schedule interpretation of results, dissemination) 4. Qualitative interviews where presence of expert by experience requested. For each person and each occasion travel estimated at £40 and subsistence at £5.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1500	1425	1950	1230	6105

Type of cost: NHS
Description: Journey Costs
Conference: True
Item Description: Conference Travel and Subsistence
Justification of Cost: Conference Travel is estimated by reviewing previous locations for conference and no allowance made for conferences which have been online or in London. Conferences included: Year 2: NIHR Academy Members Conference - travel to UK destination + accommodation Year 3: Royal College for Psychiatry International Congress - travel to UK destination + accommodation Year 4: International Meeting - Society for Psychotherapy Research - international travel + accommodation

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	150	150	700	1000

Type of cost: NHS
Description: Conference Fees
Conference: True
Item Description: Conference Fees
Justification of Cost: Conference costs are all estimated from 2024 costs Year 1: Division for Clinical Psychology Conference (UK) Year 2: NIHR Academy Members Conference Year 2: Qualitative Health Research Network Conference Year 3 Royal College for Psychiatry International Congress Year 3/4: Annual International Meeting - Society for Psychotherapy Research

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	120	400	880	600	2000

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Travel and Subsistence for Two Visits to all Six Children's Units in England
Justification of Cost: Visits to all children's units form part of the engagement strategy and orientation to the contexts to support the development of the survey and ethnography. They will be repeated in the latter stages of the project to triangulate results and share the learning from the research supporting the impact of the project. Allowance made for travel (£100) and 2 night's accommodation (£120/night)and subsistence (£25/day) for a 2-3 day visit to each unit.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2415	0	0	2415	4830

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Travel for Training Course
Justification of Cost: Travel for training courses that are part of the Training and Development Programme where attendance in person is planned Year 1: Qualitative and MIxed Method Systematic Reviews (5 days in Oxford) Year 3: AMBIT Training observation (location TBC) and attendance at Annual Forum for Quality Network for Inpatient CAMHS (1 day in UK)

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	905	0	295	0	1200

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Travel to second site for Ethnography
Justification of Cost: Travel, accommodation and subsistence for 12 weeks of observations at a non-London children's mental health unit as part of ethnography Three days each week costed at travel £100 + 2 nights accom at £120 + 3 days subsistence at £25

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	5000	0	0	5000

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Overseas Research Visit to Melbourne, Australia
Justification of Cost: Visit to inpatient units internationally where collaborators are located and where therapeutic milieu is a key aspect of approach - this will support triangulation of results, dissemination and practice and academic development. Estimated as flights to Melbourne (£1500 return) and 14 nights accommodation and subsistence

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	3580	0	3580

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Travel, Subsistence and Conference Fees Total	4940	6975	6855	4945	23715

Equipment

Type of cost: NHS
VAT relief to be applied: False
Description: Laptop
Price excluding VAT £: 1159
Expected life time: 45
Justification of Cost: Laptop specified to be able to manage large data files, audio and multimedia and have

adequate memory for this - quote created via guidance on Dell website which supports decisions as to purpose of laptop.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Year 4 (exc VAT)	Total Cost (£) (exc VAT)
	1159	0	0	0	1159

Type of cost: NHS
VAT relief to be applied: False
Description: Encrypted Voice Recorder
Price excluding VAT £: 550
Expected life time: 45
Justification of Cost: Encrypted to support confidentiality and data protection Needs to be able to manage background noise Olympus DS-9100 as recommended by other qualitative researchers

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Year 4 (exc VAT)	Total Cost (£) (exc VAT)
	550	0	0	0	550

Type of cost: NHS
VAT relief to be applied: False
Description: Encrypted Hard Drive
Price excluding VAT £: 150
Expected life time: 45
Justification of Cost: Encrypted hard drive to allow secure data back up and storage

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Year 4 (exc VAT)	Total Cost (£) (exc VAT)
	150	0	0	0	150

Type of cost: NHS
VAT relief to be applied: False
Description: NVivo Licence
Price excluding VAT £: 800
Expected life time: 45
Justification of Cost: Lifetime licence to NVivo for single user - enables it to be used on a Great Ormond Street Hospital computer to support secure data management.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Year 4 (exc VAT)	Total Cost (£) (exc VAT)
	800	0	0	0	800

Type of cost: NHS
VAT relief to be applied: False
Description: Basic Smart Phone and PAYG SIM Card
Price excluding VAT £: 400
Expected life time: 45
Justification of Cost: Smart Phone (£130) to enable phone calls, messages and WhatsApp to be used as appropriate to support PPI activities + SIM Card (£6/month)

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Year 4 (exc VAT)	Total Cost (£) (exc VAT)
	202	72	72	54	400

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Equipment Total	2861	72	72	54	3059

Consumables

Description: Stationery
Type of cost: NHS

Please describe fully: Stationery
Justification of Cost: Pens and notebooks to support field notes and recording of interviews

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	50	50	50	38	188

Description: Copying materials and posting out as needed
Type of cost: NHS
Please describe fully: Copying materials and posting out as needed
Justification of Cost: Copying information sheets Sending information for PPI support for project

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	75	75	75	56	281

Description: Poster Printing
Type of cost: NHS
Please describe fully: Poster Printing
Justification of Cost: Estimate of costs for poster printing to share results at conferences

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	50	100	75	225

Description: Printing Story Books or Leaflets
Type of cost: NHS
Please describe fully: Printing Story Books or Leaflets
Justification of Cost: Printing of leaflets or story books to support dissemination of results with children

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	200	100	300

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Consumables Total	125	175	425	269	994

Patient and Public Involvement

Description: Reading and Reviewing Materials
Type of cost: NHS
Please describe fully: Reading and Reviewing Materials
Justification of Cost: Voucher payment for time - £12.50 for 1 hour + £5 remote working costs as per NIHR and GOSH guidance Estimated at 15 hours in Year 1 and 2 and 10 hours in Years 3 and 4 10% procurement admin fee added as imposed by the supplier for the PPI vouchers

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	289	289	193	193	964

Description: Advisory Group of Experts by Experience
Type of cost: NHS
Please describe fully: Advisory Group of Experts by Experience
Justification of Cost: Voucher Payment for Time £75 for a half day of time including prep for each group 3 times / year for 6 people 10% procurement admin fee added as imposed by the supplier for the PPI vouchers

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1485	1485	1485	990	5445

Description: Thesis Committee
Type of cost: NHS
Please describe fully: Thesis Committee
Justification of Cost: Payment for one expert by experience to join thesis committee Half day of time including prep - £75 voucher payment 2 times / year 10% procurement admin fee added as imposed by the supplier for the PPI vouchers

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	165	165	165	165	660

Description: Working Groups
Type of cost: NHS
Please describe fully: Working Groups
Justification of Cost: Voucher Payment for Time Estimated at 5 people per group Each group takes 1 day and so payment for time would be £150 as per NIHR and GOSH guidance Planned groups / workshops: Year 1 - Information Sheets and Consent: 1 day Year 1/2 - Interview Schedule: 1 day Year 2 - Survey Results: 1 day Year 3 - Ethnography Results: 1 day Year 3/4 - Interview Results: 1 day Year 4 - Dissemination: 2 days TOTAL: 7 days 10% procurement admin fee added as imposed by the supplier for the PPI vouchers

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1650	825	1650	1650	5775

Description: Support for Interviews
Type of cost: NHS
Please describe fully: Support for Interviews
Justification of Cost: Voucher payment for time for expert by experience to join interviews - estimated at 15 interviews where this is requested from Year 2-3

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	413	825	0	1238

Description: PPI Funds Misc
Type of cost: NHS
Please describe fully: PPI Funds Misc
Justification of Cost: Funds to cover: -training as needed for experts by experience -conference fees for experts by experience -childcare costs or PA costs as required to support individuals involved in PPI

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	500	500	500	500	2000

Description: Refreshments to Support PPI Events
Type of cost: NHS
Please describe fully: Refreshments to Support PPI Events
Justification of Cost: Refreshments to support PPI Events as needed (e.g. to send snacks to online participants if unable to attend in person or provide tea and coffee for an advisory group)

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	100	100	100	75	375

Description: Materials to Support Creative Tasks in PPI and Qualitative Interviews
Type of cost: NHS
Please describe fully: Materials to Support Creative Tasks in PPI and Qualitative Interviews
Justification of Cost: Coloured pens, paints, paper as needed to support the engagement of young children with qualitative interviews and PPI tasks.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	100	100	75	275

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Patient and Public Involvement Total	4189	3877	5018	3648	16732

Other Direct Costs

Description: Vouchers to thank staff for completing survey
Type of cost: NHS
Please describe fully: Vouchers to thank staff for completing survey

Justification of Cost: 6 sites - 250 staff total - 25% response rate - 120 completed - £20 voucher 10% procurement admin fee added as imposed by the supplier for the PPI vouchers

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2640	0	0	0	2640

Description: Transcription
Type of cost: NHS
Please describe fully: Transcription
Justification of Cost: Estimate at 80 hours (70 hours interviews + contingency added) Quote from Verbit Go (approved supplier) indicates 1 hour, Intelligent Verbatim, 10 Days Turnaround, 2-3 Speakers = £70 + VAT 80 hours - £4,800 excl VAT

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	500	2000	2000	500	5000

Description: Translation and/or Interpreting
Type of cost: NHS
Please describe fully: Translation and/or Interpreting
Justification of Cost: Allowance to support translation of consent forms and information sheets as needed and to pay for interpreters to support participation in research as needed

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	250	500	250	0	1000

Description: Costs for animation workshop
Type of cost: NHS
Please describe fully: Costs for animation workshop
Justification of Cost: Cost calculated with support from GOSH Arts charity for a 2 day participatory workshop and film company support to produce animation from this to disseminate results.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	0	8000	8000

Description: Site Level Research Costs (x6 sites)
Type of cost: NHS
Please describe fully: Site Level Research Costs (x6 sites)
Justification of Cost: Costs for x6 Sites to perform Research Cost activities as part of the study. The overall total is £5,651.75, most costs are associated with recruitment and staff survey and so estimated to occur in Year 1 and 2 as shown.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2000	2500	1152	0	5652

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Other Direct Costs Total	5390	5000	3402	8500	22292

Training And Development

Type of cost: NHS
Description: Applicant PhD Tuition Fees
Item Description: PhD Tuition Fees
Justification of Cost: UCL PhD Tuition Fees

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	6215	6400	6592	6790	25997

Type of cost: NHS
Description: Short Courses
Item Description: Rapid Research Ethnography and Evaluation Course
Justification of Cost: Course to further develop skills in ethnography - provided by RREAL team at UCL

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	360	0	0	0	360

Type of cost: NHS
Description: Short Courses
Item Description: Essentials of Survey Research Design
Justification of Cost: Royal Statistical Society

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	833	0	0	0	833

Type of cost: NHS
Description: Short Courses
Item Description: Qualitative and Mixed Methods Systematic Reviews
Justification of Cost: University of Oxford

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	3175	0	0	0	3175

Type of cost: NHS
Description: Workshops
Item Description: Inpatient CAMHS Network Annual Forum
Justification of Cost: Attendance at workshops to enable practice development and stay connected to practice in inpatient CAMHS

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	225	0	225	0	450

Type of cost: NHS
Description: Short Courses
Item Description: Mentalisation Based Treatment - Therapy Skills
Justification of Cost: Attendance at MBT for Children and Families Modules (£800 each) Practitioner Development Group to translate to practice and enable registration (£1,000) All training by Anna Freud where this approach was developed

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	800	1000	800	0	2600

Type of cost: NHS
Description: Short Courses
Item Description: AMBIT (Adaptive Mentalization Based Integrative Treatment)
Justification of Cost: This applies the ideas of mentalisation to systems of help around particularly vulnerable, excluded, and underserved clients. This will help me connect the model I use in individual therapy to work at a system level on inpatient wards and I would aim to complete local facilitator training so that I could train teams in this approach.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	1000	0	1000

Type of cost: NHS
Description: Short Courses
Item Description: Leadership
Justification of Cost: Leading and Managing in Higher Education: A Systems-Psychodynamic Approach - Tavistock Consulting

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	0	1250	1250

Type of cost: NHS
Description: Short Courses
Item Description: Providing Good Clinical Supervision

Justification of Cost: Attend training from the Association for Psychological Therapies (APT) in clinical supervision to support me in my position as a senior clinician supervising others

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	0	500	500

Type of cost: NHS
Description: Short Courses
Item Description: Framework Method for Data Analysis
Justification of Cost: Analysis of Qualitative Data - Social Research Association

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	440	0	0	440

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Training And Development Total	11608	7840	8617	8540	36605

Total Direct Costs

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Total Direct Costs	118898	113724	114174	93294	440090

Indirect costs

HEI Indirect costs
No costs have been added

Commercial Indirect Costs
No costs have been added

Other Partner Organisation Indirect costs
No costs have been added

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Total Indirect Costs	0	0	0	0	0

Research Costs Requested From Funder

	Direct costs	Indirect costs/Overheads	Amount requested
Total Higher Education Institution Costs (80%)	0	0	0
Total Higher Education Institution Costs (100%)s	0	0	0
Total Commercial Costs	0	0	0
Total Other Parther Organisation Costs	0	0	0
Total NHS Research Costs	440090	0	440090

Total Research Costs Requested From Funder	440090	0	440090

Summary

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Salaries	**89785**	**89785**	**89785**	**67338**	**336693**
Travel-subsistence-and-conference-fees	**4940**	**6975**	**6855**	**4945**	**23715**
Equipment	**2861**	**72**	**72**	**54**	**3059**
Consumables	**125**	**175**	**425**	**269**	**994**
Patient-and-Public-Involvement	**4189**	**3877**	**5018**	**3648**	**16732**
Other-Direct-Costs	**5390**	**5000**	**3402**	**8500**	**22292**
Training And Development	**11608**	**7840**	**8617**	**8540**	**36605**
HEI-Indirect-costs	**0**	**0**	**0**	**0**	**0**
Commercial-Indirect-Costs	**0**	**0**	**0**	**0**	**0**
Other-Partner-Organisation-Indirect-costs	**0**	**0**	**0**	**0**	**0**
Total	**118898**	**113724**	**114174**	**93294**	**440090**

NHS Support Costs

Cost Item:	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Site Level Service Support Costs (x6 sites)	628	622	50	0	1300.0

Justification:
Site Level Service Support Costs (x6 sites): Site Level Service Support Costs (x6 sites) The overall total is £1,300.37 £1128 for database search included across Year 1 and 2 (628 in Year 1 and 500 in Year 2) £172.37 is for Interviews with Child + Parent and included in Year 2 (£122) and Year 3 (£50).

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
NHS Support Costs Total	628	622	50	0	1300

Have you discussed and agreed these costs with the Lead Network?	Yes

NHS Treatment Costs

No costs have been added

Have you discussed and agreed the NHS costs with the Lead Trust?	N/A

Is the patient/service user care being provided different from the usual standard treatment for the condition?
N/A

Usual Treatment Costs

No costs have been added

Excess Treatment costs/(savings)

	Cost per patient(£)	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Total NHS Excess Treatment costs/(savings)	0	0	0	0	0	0

	Total (£)
NHS Support Costs requested from networks	1300
NHS Excess Treatment costs/savings	0
TOTAL NHS SUPPORT AND TREATMENT COSTS	1300

Total Funding Required (including non-research costs)

	Total (£)
Total Research Costs Requested from Funder	440090
Total NHS (Support and Excess Treatment) Costs	1300
Total Funding Required	441390

10. Uploads

Please upload the following documents, if necessary.

When uploading a document to your application, please ensure that it is in an appropriate file format. It is not possible to upload JPEG images, and with the exception of the SoECAT Form, which should be uploaded as an Excel spreadsheet, all files should be uploaded in Adobe PDF or Word DOC/DOCX format. Please refer to the Guidance Notes for further information on the attachments required.

(References - References.pdf) is included as an appendix within this file.

(Research Timetable - Project Plan DCAF.pdf) is included as an appendix within this file.

(SoECAT Template - 22BO21 - IRAS 340055 SoECAT, Therapeutic Milieu Inpatient CAMHS (VALIDATED 28May2024).xlsx) is included as an appendix within this file.

11. Participants and Signatories

Participants
You are required to supply the name and email address (if not already registered on the ARAMIS application system) of each required participant. Everyone included in this section will be acting as a 'participant' in your application and must agree to be named as such.

By confirming participation, participants are acknowledging their involvement and input into this application, and are agreeing to be involved, as indicated, in any resultant fellowship.

Once the 'Administrative Authority or Finance Officer' participant has confirmed their participation, they will be able to access the 'Detailed Budget' section of the application form. You should collaborate with your 'Administrative Authority or Finance Officer participant to ensure that this section is completed satisfactorily.

You must ensure that all participants are happy for your application to be submitted before you submit it.

It is permissible for a single individual to serve as both a signatory and a participant, but they will need to be added separately under each heading.

Named participants must confirm their participation on your application before you will be able to select the 'SUBMIT' option. Please refer to the Submission Process Flow Diagram and the Applicant Guidance Notes for further information.

Primary Doctoral Supervisor

Doctoral Primary Supervisor Participant 1

Title	Professor
Forename(s)	Julian
Surname	Edbrooke-Childs
Position	Professor

Additional Academic Supervisor(s)

Supervisor Participant 1

Title	Professor
Forename(s)	Nick
Surname	Midgley
Position	Lecturer

Supervisor Participant 2

Title	Dr
Forename(s)	Cecilia
Surname	Vindrola
Position	

Practice Supervisor(s)

Practice Support Participant 1

Title	Dr
Forename(s)	Sinead
Surname	Marriott
Position	

Practice Support Participant 2

Title	Dr
Forename(s)	Tara
Surname	Murphy
Position	

Administrative Authority or Finance Officer

Administrative Authority or FO Participant

Title	Mr
Forename(s)	Kevin
Surname	Samuel
Position	Finance Manager

Signatories
You are required to supply the name and email address (if not already registered on the ARAMIS application system) of each required 'Head of Department / Senior Manager' signatory. Once their contact details have been entered below, these signatories can be invited to log in and confirm their participation in your application.

Once the nominated signatories have confirmed their participation, they can access and complete the required 'Host Organisation Statement of Support' in the 'Training & Development and Research Support' section of the application form.

Your signatories must confirm their participation in your application and complete the section they are responsible for before you can select the 'submit' option.

Signatories must also approve your application after you select the 'submit' option but BEFORE the application submission deadline.

Signatory approval from your 'Head of Department / Senior Manager' signatories will result in the application being fully submitted to the NIHR. All parties (applicant, participants and signatories) will be notified of this via an automated system-generated email.

The NIHR will not accept your application unless fully approved by your signatories before the 1 p.m. deadline on Tuesday 11th June 2024.

Heads of Departments / Senior Managers

Head of Department or Senior Manager 1

Title	Dr
Forename(s)	Helen
Surname	Griffiths
Position	

Head of Department or Senior Manager 2

Title	Professor
Forename(s)	Stephen
Surname	Pilling
Position	Professor

12. Acknowledgement, Review and Submit

Conflict checks: Please declare any conflicts or potential conflicts of interest that you may have in undertaking this research, including any relevant, non-personal & commercial interest that could be perceived as a conflict of interest.
I have no conflicts of interest in undertaking this research.

By completing this form, the programme reserves the right to share, in confidence, details of your application with other parts of the NIHR and with the Department of Health and Social Care.

Agreement to Terms and Conditions: I confirm that I have secured all necessary licences and ethical approvals in relation to the research and will abide by the terms of those licences and approvals in carrying out the research. I confirm that I have read the NIHR Carbon Reduction Guidelines and, where possible, have taken steps to reduce the carbon emissions generated by this research. I confirm that all material within this application, with the exception of sections that other participants are required to complete, is my own work and I have taken steps to ensure that no parts of the application have been inappropriately plagiarised. I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time lay down according to the NIHR research governance framework. I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time laydown according to the UK Policy Framework for Health and Social Care Research. In completing this, I declare that, to the best of my knowledge, the information provided in this application, is true, accurate and complete. I have read the Guidance Notes and agree to accept the process by which an application is assessed and agree to abide by the conditions under which an award may be granted.
☒Confirmed

Appendices

1. References.pdf
2. Project Plan DCAF.pdf
3. 22BO21 - IRAS 340055 SoECAT, Therapeutic Milieu Inpatient CAMHS (VALIDATED 28May2024).xlsx

Appendix #1: References.pdf

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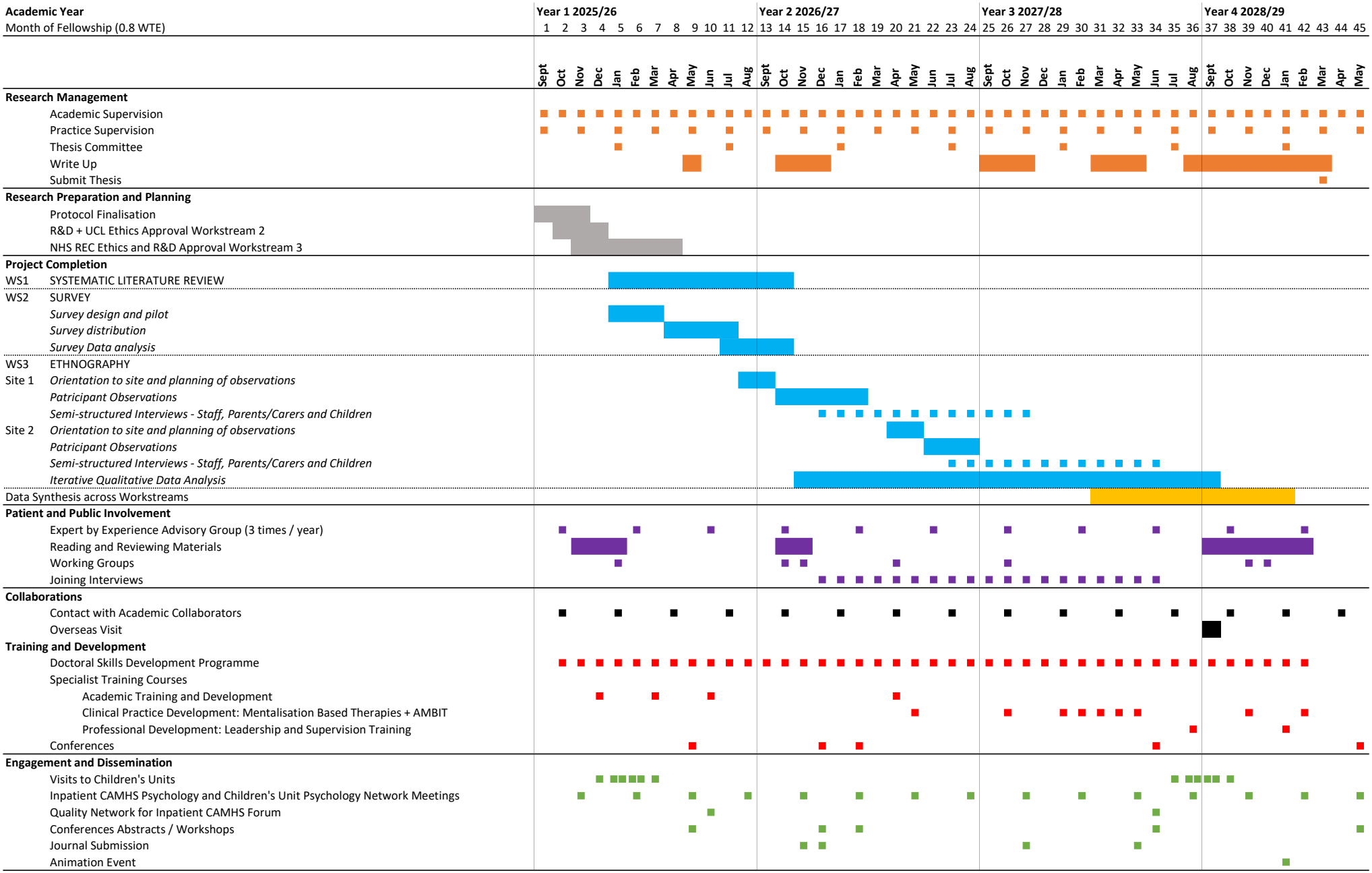
Appendix #2: Project Plan DCAF.pdf

DCAF TIMETABLE
DR DAWN CUTLER
DCAF SEPTEMBER 2025 - MAY 2029 (45 MONTHS)



Appendix #3: 22BO21 - IRAS 340055 SoECAT, Therapeutic Milieu Inpatient CAMHS (VALIDATED 28May2024).xlsx